Adjuvant abemaciclib + endocrine therapy

NCCP 00619 · Draft — independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Your practical treatment plan

Abemaciclib is taken twice daily continuously, with endocrine treatment, for up to two years if appropriate. A 28-day block is shown for orientation; there is no planned tablet-free week. Endocrine treatment usually continues longer.

STEP 1

Before starting

Confirm the exact tablet strength, endocrine partner, blood-test and ECG plan where relevant.

STEP 2

At home

Take tablets whole at the prescribed times. Abemaciclib is continuous twice daily; ribociclib has a planned 7-day break. Follow the schedule for this card only.

STEP 3

If a dose is missed or vomited

Do not double up or take a replacement dose. Follow the medicine instructions and ask the oncology pharmacist if uncertain.

STEP 4

Before the next supply

Confirm symptoms and required test results have been reviewed. Ask whether the prescription changes; do not restart a held medicine yourself.

▼ Interactions and storage

Check all new medicines and supplements with oncology/pharmacy, avoid grapefruit, store as labelled and keep away from children. Return unwanted tablets to the pharmacy.

▼ Endocrine and ovarian-suppression visits

An aromatase inhibitor is generally a daily tablet. Fulvestrant, when selected for metastatic treatment, has loading injections on Days 1, 15 and 29 then monthly. Ovarian-suppression injections have their own prescription and visit schedule.

▼ No infusion pump

The CDK4/6 medicine is taken orally. An implanted port or infusion pump is not required for these tablets; other treatment may have separate access needs.

Sources: NCCP 00619 v4a · official regimen · EMA · Verzenios product information and patient leaflet. Draft — independent clinical, pharmacy and patient/PPI review pending.